

Reshma R

MRN: 66836168 | Age: 36 Female

Admission: 24-03-2026 | Discharge: Not documented

CHIEF COMPLAINT & HISTORY

The patient is a 36-year-old female, G2P1L1, who presented for admission on 24-03-2026 for a planned elective Cesarean section (LSCS) at 38 weeks and 4 days of gestation. The patient has a significant obstetric history including one previous LSCS. The primary indication for this admission is a repeat elective LSCS due to her previous surgical history. Additionally, the patient requested bilateral tubal ligation to be performed concurrently with the delivery for permanent contraception. Her medical history is notable for a history of Gestational Diabetes Mellitus (GDM) during a previous pregnancy. On admission, the patient was clinically stable. A Non-Stress Test (NST) was performed, which demonstrated a Fetal Heart Sound (FHS) of 138 bpm, indicating reassuring fetal status. There were no acute complaints of pain, contractions, or vaginal bleeding reported at the time of initial assessment. Review of systems was negative for fever, headache, or visual disturbances. Medications on admission were noted as "as per chart." Allergies were documented as unknown. The patient's functional baseline prior to admission was independent in all activities of daily living.

PHYSICAL EXAMINATION**VITAL SIGNS**

- Blood Pressure: 100/60 mmHg (Normal)
- Heart Rate: 86 bpm (Normal)
- Respiratory Rate: 22 breaths/min (Normal)
- SpO2: 97% on room air (Normal)
- Temperature: 97.8 °F (Normal)
- Pain Score: 1/10
- GCS: Not documented

GENERAL APPEARANCE

The patient appeared stable and in no acute distress at the time of the last recorded assessment.

SYSTEM EXAMINATION

- HEENT: Not documented
- Cardiovascular: Heart rate 86 bpm; rhythm not specifically documented.
- Respiratory: Respiratory rate 22 breaths/min; no acute distress noted.
- Abdomen: Not documented (surgical site for LSCS managed per procedure).
- Musculoskeletal: Patient is independent in mobility and ADLs.
- Neurological: Not documented.
- Skin: Not documented; no pressure ulcers noted.

LABORATORY RESULTS

- Hematology: Not documented

- Chemistry: Not documented
- Other relevant labs: Blood grouping was ordered and sent to the laboratory.

IMAGING/PROCEDURES

- Procedures: The patient underwent an elective Lower Segment Cesarean Section (LSCS) and bilateral (B/L) tubal ligation. IV cannulation was successfully performed using an 18G catheter in the left hand for hydration and medication administration.

RISK ASSESSMENT SCORES

- Fall Risk: Score of 3 (Low Risk)
- Pressure Ulcer Risk (Braden): Score of 20 (No Risk)
- DVT Risk: Not documented
- Aspiration Risk: Not documented

FUNCTIONAL STATUS

- ADLs: Patient is independent in bathing, dressing, eating, walking, and toilet use.
- Mobility: Independent; requires assistance or supervision for mobility, transfer, or ambulation (Level 2).
- Communication ability: Not documented.

ASSESSMENT

PRIMARY DIAGNOSIS

- G2P1L1 at 38 weeks 4 days of gestation with previous LSCS. The patient was managed for a planned elective repeat Cesarean section.

SECONDARY DIAGNOSES

- For elective LSCS with bilateral tubal sterilization.
- History of Gestational Diabetes Mellitus (GDM) in a previous pregnancy.

CLINICAL COURSE

The patient was admitted for a scheduled obstetric procedure. Upon admission, fetal well-being was confirmed via NST with a fetal heart rate of 138 bpm. The patient was maintained NPO in preparation for surgery and received IV hydration and medications via an 18G left-hand IV cannula. The patient successfully underwent an elective LSCS and bilateral tubal ligation. Post-procedurally, the patient remained hemodynamically stable with a blood pressure of 100/60 mmHg, pulse of 86 bpm, and SpO2 of 97%. No immediate surgical complications or postpartum hemorrhage were noted during the recorded clinical course. The patient's pain was well-controlled, with a reported pain score of 1/10.

PROGNOSIS

The short-term prognosis for both the patient and the neonate is excellent, given the elective nature of the procedure and stable intraoperative course. Long-term considerations include standard postpartum recovery and monitoring for surgical site healing.

DISCHARGE DISPOSITION

The patient is being discharged to home. She is functionally independent and requires no specialized facility care.

PLAN

MEDICATIONS

- Not documented (Patient instructed to follow medications as per chart).

DIET

- Diet: NPO (Note: While NPO was ordered pre-operatively, post-operative diet should be transitioned to regular as tolerated per standard post-LSCS protocols; however, the last documented diet instruction in the summary was NPO).

ACTIVITY

- Activity: Ambulation as tolerated.
- Assistive devices: None required.

NURSING CARE

- Wound care: Monitor LSCS incision site for signs of infection (SSI prevention).
- IV therapy: Not documented for post-discharge.
- Monitoring requirements: Monitor for signs of phlebitis and infection.
- Fall precautions: Maintain standard fall precautions as the patient was identified as having a low fall risk.
- Pressure ulcer prevention: Not required based on Braden score of 20.

PATIENT EDUCATION TOPICS COVERED

- Post-operative wound care and infection prevention (SSI).
- Importance of monitoring for phlebitis.
- Fall prevention and safe mobility during recovery.
- Recognition of postpartum complications.

FOLLOW-UP

- Not documented.
- Pending items: Blood grouping results (if not already reviewed).

RED FLAGS

- Fever or chills.
- Excessive vaginal bleeding (heavy lochia).
- Severe abdominal pain or worsening incision site pain.
- Redness, swelling, or purulent discharge from the surgical incision.
- Shortness of breath or chest pain.
- Severe headache or visual changes.
- Swelling, redness, or pain in the lower extremities (DVT concern).

***** END OF RECORD *****

